

The National Project "Long And Active Life" As A Tool For Sustainable Development Of Regional Healthcare

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Abstract. This article examines the current state and trends of key healthcare system indicators in the constituent entities of the North Caucasus Federal District (NCFD) in the context of the implementation of the national project "Long and Active Life" in 2025. Using statistical data, a quarterly analysis was conducted of indicators characterizing the development of primary healthcare, staffing, preventive care, and the results of federal projects to combat cardiovascular diseases, cancer, and diabetes. Significant interregional differentiation in indicator trends was identified, reflecting differences in institutional sustainability and management practices across the NCFD. It is shown that despite positive shifts in healthcare accessibility, preventive activity, and the introduction of modern medical technologies, systemic limitations associated with staffing shortages and the uneven effectiveness of specialized medical care persist. A conclusion is drawn regarding the need to transition from a predominantly quantitative approach to achieving target indicators to a comprehensive healthcare development management model based on regional specifics and long-term human resource planning.

1 Introduction

The current stage of socioeconomic development in the Russian Federation is characterized by a shift in public policy emphasis toward prioritizing the development of human capital as a key resource for sustainable growth and national security. In this context, the healthcare system is acquiring strategic importance, as it creates the fundamental conditions for preserving the population, increasing life expectancy and quality of life, strengthening the health of citizens, and realizing their labor and demographic potential [1, 2]. At the same time, the regional healthcare system in the Russian Federation faces a number of systemic problems that negatively impact the accessibility and quality of medical services. Among the most significant of these are limited resource provision for medical organizations, the increasing prevalence of socially significant diseases, a shortage of primary healthcare personnel, and insufficient salaries for healthcare workers, which reduces their professional motivation and the sustainability of the workforce [3, 4].

The relevance of revising priorities in social and economic policy was enshrined at the highest state level. The Decree of the President of the Russian Federation of May 7, 2024 "On the national development goals of the Russian Federation until 2030 and for the future until 2036" defined a new system of national goals, which focuses on strengthening the health of citizens, increasing the level of well-being and comprehensive support of families [5]. For the practical implementation of the designated strategic guidelines, the national project (NP) "Long and Active Life" was developed, the launch of which is scheduled for 2025. The implementation of the project involves systemic transformations in the healthcare sector aimed at increasing the availability and quality of medical care, strengthening human resources, developing preventive medicine and achieving target indicators by 2030 [6-9]. In this regard, the analysis of the current state of medical care in the regions of the North Caucasus Federal District is of particular importance, allowing us to identify the initial conditions, disproportions and potential risks of achieving the set goals [10, 11].

The aim of the study is to analyze the main indicators of the healthcare system of the North Caucasus Federal District for 2025 in the context of the implementation of the national project "Long and Active Life".

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2 Research methodology

To achieve this goal, the indicators of the health care system reflecting the results of the implementation of the Long and Active Life non-profit partnership were analyzed from the open register of the Unified Medical Information System (UIISS) of the Federal State Statistics Service in the Republic of Dagestan (RD), the Republic of Ingushetia (RI), the Chechen Republic (CR), the Kabardino-Balkarian Republic (KBR), the Karachay-Cherkess Republic (KCR), the Republic of North Ossetia-Alania (RSO-A), Stavropol Krai (SK) and the Russian Federation for 2025 on a quarterly basis (Quarter I – January, February, March; Quarter II – April, May, June; Quarter III – July, August, September; Quarter IV – October, November, December) [12]. Using Microsoft software Using Excel 2016 and SPSS "Statistics" relative deviations were calculated in percentage terms, graphs and diagrams were constructed.

3 Results and Discussions

The results of the conducted study on the implementation of the national project "Long and Active Life" in the constituent entities of the North Caucasus Federal District in 2025 are generally consistent with previously identified trends in the development of the regional healthcare system, primarily in terms of staffing and the sustainability of the primary care sector. Thus, the data obtained in this study on the heterogeneous dynamics of the provision of physicians and mid-level medical personnel, as well as the persistent regional disparities, complement and develop the conclusions made in the study devoted to the analysis of the human resources potential of the healthcare system of the North Caucasus Federal District for 2020-2024. In particular, it was previously shown that in most constituent entities of the North Caucasus Federal District, a stable trend towards a decrease in the provision of medical organizations with outpatient physicians and emergency medical specialists persists, with the presence of local positive shifts due to the implementation of individual regional personnel programs [13]. Similar patterns can be observed in the analysis of 2025 indicators, where, despite the overall increase in the availability of medical personnel in a number of regions, negative dynamics are recorded in the Republic of Dagestan and the Stavropol Territory, indicating a persistent structural shortage of personnel and the limited effectiveness of mechanisms for their retention.

Of particular importance in the context of comparing the results is the analysis of mid-level personnel. As demonstrated in a previously published study, mid-level medical personnel form the foundation of the healthcare system's sustainability; however, their numbers and structure are subject to the greatest fluctuations due to demographic aging, professional migration, and the profession's insufficient social attractiveness [13, 14]. The data obtained in this study support this position and indicate the need to strengthen targeted support measures for mid-level medical personnel within the framework of the federal "Medical Personnel" project.

4 Conclusions

The study provided a comprehensive assessment of the implementation of the national project "Long and Active Life" in the North Caucasus Federal District in 2025 and identified key trends, constraints, and structural features of the regional healthcare system's development. The results indicate that the implementation of federal projects within the national project is contributing to positive shifts in access to healthcare, the development of preventive measures, and the adoption of modern medical technologies, including digital tools. However, these effects are uneven and, in some cases, unstable.

An analysis of primary healthcare and specialized medical care modernization indicators revealed that, despite positive trends in certain indicators, significant regional disparities persist due to staffing levels, management practices, and the institutional sustainability of healthcare organizations. Particularly noteworthy is the shortage of healthcare personnel, primarily physicians and nurses, which continues to systemically impact the accessibility and quality of healthcare in several regions of the North Caucasus Federal District, despite the implementation of the federal "Medical Personnel" project. In specialized medical care, an increase in early cancer detection rates was found to be coupled with persistently unfavorable trends in hospital mortality from acute cardiovascular conditions, highlighting the need for further improvement of patient routing systems, the development of regional vascular centers, and increased access to high-tech medical care. An analysis of preventive and behavioral risk factors confirms the effectiveness of healthy lifestyle promotion measures but

reveals persistent regional disparities requiring targeted adjustments to preventive programs, taking into account the sociodemographic characteristics of the population.

Taken together, the obtained results confirm the need to shift from a predominantly quantitative approach to achieving indicators to a comprehensive healthcare development management model based on regional specifics, demographic burden, and long-term workforce planning, as previously substantiated in studies on human resource development in regional healthcare. Promising areas for the further implementation of the "Long and Active Life" non-profit partnership in the North Caucasus Federal District include strengthening targeted human resources policies, integrating quality of care indicators into the performance assessment system, developing interlevel coordination, and transitioning to management focused on sustainable structural change.

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